

Larkspur Health Network Compliance Assessment: Patient Records Access and Privacy Controls

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Executive Summary

This compliance assessment evaluates whether access to patient records across Larkspur Health Network meets the network's privacy policy and the applicable health privacy regulations. The assessment covered the period from 1 September 2025 to 31 January 2026 and was carried out by the Office of Compliance and Privacy with support from the information security team.

The assessment found that a large number of accounts belonging to former staff remain active, that access to a sensitive record category at the network's flagship hospital is wider than the roles require, and that privacy training completion is below the network standard. Emergency access review is working well.

Four findings and three recommendations are presented. The Chief Compliance Officer has accepted all three recommendations. The overall assessment rating is that the network is partially compliant.

Background

Larkspur Health Network operates three hospitals and nineteen outpatient clinics and employs about 6,100 staff. Access to clinical applications is governed by Sentinel Access Control, the identity and access management system that provisions accounts and enforces role-based permissions. Accounts are created from the human resources system but deactivation is a manual step performed by department managers.

The Willow Creek Medical Center is the network's flagship facility, a 380-bed hospital that houses the regional cancer centre. Oncology records at the centre are classed as a sensitive record category and access is meant to be limited to the treating teams.

The Privacy Compliance Program is the network-wide program that sets privacy policy, delivers annual training and investigates complaints. The program requires 95 percent of staff to complete privacy training each year.

The assessment compared the full account list with the human resources termination records, reviewed access to oncology records for all 1,940 users with any oncology permission, examined training records and sampled 50 break-glass access events.

Findings

Finding 1: 412 user accounts in Sentinel Access Control belonged to staff who had left the organization more than 30 days earlier. Severity: High. Sixty of these accounts had been used to log in after the termination date, in most cases by the departing employee working a final shift that was not reflected in the termination record. Nine logins could not be explained and have been referred for investigation.

Finding 2: Access to oncology records at the Willow Creek Medical Center was granted to 27 users whose role did not require it. Severity: High. The users included administrative staff in other departments and two contractors. There is no evidence that any of the 27 users viewed oncology records inappropriately, but the access should not have existed.

Finding 3: Annual privacy training completion under the Privacy Compliance Program reached 91 percent, below the 95 percent standard. Severity: Medium. Completion was lowest among night shift and casual staff, who reported difficulty finding time for the training during working hours.

Finding 4: Break-glass emergency access events were reviewed by the privacy office within 24 hours in every sampled case. Severity: Low. All 50 sampled events had a documented review and a clinical justification. The assessment notes this as an effective control.

Recommendations

Recommendation 1: Automate account deactivation in Sentinel Access Control from the human resources termination feed. Priority: High. The integration should deactivate accounts at the end of the last working day recorded in the human resources system, with a same-day exception process for late changes.

Recommendation 2: Remove oncology record access from users at the Willow Creek Medical Center whose role does not require it and re-certify access quarterly. This action should be completed without delay, since every additional day of unnecessary access adds to the exposure described in Finding 2. Department managers should confirm each user's continuing need for access as part of the quarterly re-certification.

Recommendation 3: Bring privacy training completion to 95 percent by 30 June 2026 through manager escalation. Priority: Medium. Managers should receive a monthly list of staff with incomplete training, and the training should be made available on the ward terminals so that night shift and casual staff can complete it during quiet periods.

Financial Impact

The estimated maximum regulatory penalty exposure arising from the access findings is \$1.6 million, based on the penalty tiers in the applicable regulations. This is a ceiling rather than an expected cost, and no regulatory action has been initiated.

The access control automation cost for integrating the human resources feed with Sentinel Access Control is quoted at USD 84,000. The network's information technology department has confirmed that the work can be completed within the current financial year.

Appendix

Appendix A: Management response. The Chief Information Officer accepts Recommendation 1 with a target date of 31 May 2026. The Willow Creek Medical Center Chief Medical Officer accepts Recommendation 2 and confirms that the 27 accounts were remediated on 2 March 2026. The Director of Learning accepts Recommendation 3 with a target date of 30 June 2026.

Appendix B: Methodology. The account comparison used the human resources termination extract as at 31 January 2026. Oncology access was reviewed against the role matrix approved by the cancer centre in 2025. The break-glass sample was drawn at random from 1,412 events in the assessment period.

Appendix C: Distribution. This report is distributed to the Board Quality and Compliance Committee, the Chief Executive, the Chief Compliance Officer and the Chief Information Officer.